Planning and Evaluating Community Health Services

Goal: Increase skills and knowledge on planning and evaluating community health programs

Specific objectives:

- Describe the concept of planning in community health
- Explain goal setting and planning process of community health programs
- Describe the concept of evaluation in community health

Community Health Planning - Community health planning is a systematic process of identifying the health needs of communities and developing evidence-based recommendations and strategies to promote and improve the health of individuals and communities.

Goal - The goal of planned approach to community health services is to increase the capacity of communities to plan, implement, and evaluate comprehensive, community-based health programs targeted toward priority health problems. Planned approach to community health helps in:

- Systematic implementation of activities
- Establishing a health working teams
- Define key data to be collected and used
- Set health priorities, and design and evaluate interventions
- Identify and address priority health problems or special population to be addressed
- It can also be adapted and used by existing organizational and planning structures in the community.

A planning cycle in the development of a community health program

- A planning cycle that includes:
- A situation analysis,
- Formulation of objectives,
- Selection of strategies,
- Development of an operational plan,
- Implementation and evaluation, which lead, in turn, to a new situation analysis.

Using data to identify health problems

- On the basis of information collected during phase II, the community group needs to identify health priorities and determine which ones it wants to address first.
- As the community group reviews different types of data, it develops criteria for determining priorities within the data. For example, as members review mortality data, they might consider something a priority if:
    - It causes a large number of deaths.
    - It is a leading cause of years of potential life lost.
    - It is a greater problem in your community than in the state or nation.

- By matching the mortality data to the criteria, the group would identifying priorities for each of the other types of data

Choosing Health Priorities

Most communities do not have the resources to address all of their health problems and target groups at once. They must set priorities and plan to address some problems initially and others over time. We recommend that the community group select only one or a limited number of health problems to better focus resources in a comprehensive manner. The following are priority setting techniques:

- Problem/Preference Ranking - is a participatory technique that allows analyzing and identifying problems or preferences *stakeholder* share in order to implement adequate improvements and solutions in their community and area. First, the *stakeholders* have to decide which the most important problems they face in their community are. Afterwards, the participants have to rank these problems/preferences in regards to their importance. The result of this method provides the starting base for discussions on possible solutions to the priority problems.

- A more systematic technique is called "pairwise ranking" and uses cards to represent the different problems. The facilitator shows the "problem cards", two at a time, each time asking, "Which is the bigger problem?" As the participants make the comparisons, the results are recorded in a matrix.

The criteria attached to make up a choice are used to consider in the community action plan. Community action plans are developed on the basis of people's preferences. The problems, solutions technical inputs are arranged on the interests of the users.

Advantages

- Preference/Problem ranking method helps to quickly get a good idea of what people think are the priority problem or preferences
- Preference/Problem ranking is probably the easiest method to use and to learn more about commonly shared problems and priorities
- The criteria developed out of the ranking can be used for the community action plan of the area

Disadvantages

- It requires time and some logistic efforts to bring all relevant stakeholders together in order to rank the preferences and problems
- It may not be possible to react to all the problems discussed

As the community group works to set priorities, it must examine the intervention focus and its capacity to address one or more problems and agree on which problem or problems to address first.

Identifying which problem(s) to address first

When examining priority behavioral risk factors, you might consider the following:

- **Importance**—evidence that the behavior change will make a difference….eg. How widespread is the behavior? - How serious are the health consequences? How close is the connection between the behavior and the health problem?
- **Changeability**—evidence that the behavior is amenable to change ….e.g. Is the behavior still in developmental stages? Is the behavior only superficially tied to lifestyle? Has the behavior been successfully changed in other programs? Does the literature suggest that the behavior can be changed?

Many community groups find it helpful to use the nominal group process or other techniques to identify the top three to five health problems. These three to five problems are then analyzed further to determine which to address first. Some issues to consider when ranking problems include:

- Legal and economic factors,
- Political viability of the intervention,
- Possibility of continued funding,
- Probability of quick success,
- Ability to build on community strengths,
- Level of public concern.

Goal setting

Goal setting involves establishing specific, measurable, achievable, realistic and time-targeted (S.M.A.R.T) goals. Setting goals helps people work towards their own objectives. Goal setting features as a major component of personal development literature.

- Characteristics of a good goal:
  - **S**pecific - When setting goals, they should answer the highly specific questions of who, what, where, when, and why.
  - **M**easurable - In order for us to track our progress, goals should be quantifiable.
  - **A**ttainable - It is important to evaluate your situation honestly and recognize which goals are realistic, and which are a little far-fetched.
  - **R**elevant - Is this goal relevant to your life? Does this match my needs?
  - **T**ime-related - Setting a "due date" to meet goals not only keeps you on track, but it prevents pesky daily roadblocks from getting in the way.
- Facts about goal setting include:
  - Specific, realistic goals work best
  - It takes time for a change to become an established habit
  - Repeating a goal makes it stick
  - Pleasing other people doesn't work
  - Roadblocks don't mean failure
- How to set goals:
  - *Think about the "big picture*." - Ask yourself some important questions about what you want for your life.
  - *Break the "big picture" down into smaller and more specific goals* - Consider areas of your life that you either want to change or that you feel you would like to develop with time. Begin to ask yourself specific questions about what you'd like to achieve in each area and how you would like to approach it within a five year timeframe.
  - *Make each goal a positive statement* **-** Using positive statements is a direct way to affirm your commitment to completing your goals.
  - *Set priorities* **-** At any given moment, you have a number of goals all in different states of completion. Deciding which goals are more important, or time-sensitive, than others is crucial. If you have a college interview scheduled on the same day as a training session for your big half-marathon, the interview would take precedent as it is far more time-sensitive, and probably more important.
  - *Keep track of your progress* **-** Journaling is a great way to keep track of both personal and professional progress. Checking in and acknowledging the progress made towards a certain goal is key to staying motivated.
  - *Reward your accomplishments* **-** Acknowledge when you have reached goals and

allow yourself to celebrate accordingly.

Writing goals and objectives

- Goals are broad, abstract statements of intent that help create a vision of what you are striving to accomplish.
- Objectives are measurable, specific statements that lead toward program goals and define what change the community will try to achieve.
- The importance of both in anchoring the community health-planning process cannot be emphasized enough, for vague goals and objectives are likely to yield scattered, unfocused efforts.
- Each objective should answer these questions:
    - Who will receive the intervention? (Whose health is its focus?)
    - What health benefit should these persons receive?
    - How much of that benefit should they receive?
    - By when should it be achieved?

For example:

- *By 1998, the prevalence of smoking among county residents aged 18 years and older will be reduced by 15% from 25% (BRFSS 1991 baseline) to 21%.*
- *Our goal is to reduce the number of premature deaths due to heart disease in our community.*
- *By 1998, the rate of fatal injuries among county residents caused by drinking and driving will be reduced by 15% from 7/1,000 to 5/1,000.*

Objectives are active, working tools and not merely academic exercises. An objective:

- Specifies a single key result.
- Specifies a target date.
- Is specific and quantitative.
- Specifies what and when, not why and how.
- Is readily understandable to those involved.
- Is realistic, attainable, yet a challenge.
- Provides limits to expenditures of time and effort.
- Identifies criteria for evaluating achievement.
- Provides orientation to cooperating agencies in the community.

Community health planning uses two types of objectives to clarify community goals:

- Behavioral objectives—for the leading behaviors that contribute to death or disability and are the focus of the intervention
- Intervention objectives—for the interventions you wish to undertake

Writing behavioral and intervention objectives

- Behavioral objectives refer to those behavioral risk factors that contribute to the cause of death specified in your community goals.
- The intervention objectives refer to the intervention activities you plan to undertake.

- The community goal is more general, the behavioral objectives are more specific, and the intervention objectives the most specific. Using the above example, the hierarchy of objectives would include the following:

Community goal:
Our goal is to reduce the number of premature deaths due to heart disease in our community.

Behavioral objectives:
- *By 2000, the prevalence of county residents who smoke will be reduced by 20%, from 32% (1994 BRFSS data) to 25.6%.*
- *By 2000, the prevalence of physically inactive adults in our county will be reduced by 15%, from 38% (1994 BRFSS data) to 32%.*

Intervention objectives:
- *By January 1995, 20% of participants in a "Quit and Win" smoking cessation contest will still be nonsmokers one year after the contest (January 1994).*
- *By January 1996, the rate of onset of smoking among county school students, grades 6-9, will be reduced by 20% from 158/1,000 (1993 YRBS data) to 128/1,000.*
- *By January 1995, the county school board will set policy that allows the community to use school playing fields for evening and weekend recreational activities.*
- *By July 1996, 5 companies that employ 50 or more workers in the community will provide access to programs that address physical activity, good nutrition, and cessation of tobacco use.*

Community health planning process
Although plans can be adapted to various health problems and communities, the phases of the process remain the same. Thus, once the mechanisms of the planning process are in place, only a few modifications are needed to address additional health issues. Phases can be repeated as new health priorities are identified, new target groups are selected, or new interventions are developed. The activities within phases may overlap as the process is carried out. Each of the five phases that constitute plan is described hereafter.

Step I: Mobilizing the community - Mobilizing the community is an ongoing process that starts in step I as a community organizes to begin planning and continues throughout the process. In phase I:
- The community to be addressed is defined,
- Participants are recruited from the community
- Partnerships are formed
- Demographic profile of the community is completed

Step II: Collecting and organizing data - Step II begins when the community members form working groups to obtain and analyze data on mortality, morbidity, community opinion, and behaviors. These data, obtained from various sources, include quantitative data (e.g., vital statistics and survey) and qualitative data (e.g., opinions of community leaders).

Step III: Choosing health priorities - During this phase, objectives related to the health priorities are set. The health priorities to be addressed initially are selected.

Step IV: Developing a comprehensive intervention plan - Using information generated during steps II and III, the community group chooses, designs, and conducts interventions during phase.

Step V. Build on existing services **-** In order to prevent duplication and to integrate the existing services, the community group identifies and assesses resources, policies, environmental measures, and programs already focused on the risk behavior and to the target group.

Step VI: Evaluating plans **-** Evaluation is an integral part of the planning process. It is ongoing and serves two purposes: to monitor and assess progress during the five phases of planning and to evaluate interventions. The community sets criteria for determining success and identifies data to be collected.

Community planning materials

This Concept Guide is part of a three-part package of materials designed for the local coordinator, the person who initiates the planning process within a community. This local coordinator will:

- Have major coursework and experience in health education and community health promotion.
- Be able to adapt the planning process to meet the needs of the community.
- Serve as facilitator of this community-based process by working with a broad-based community group and ensuring community ownership.
- Use expertise and resources at the community, state, and federal levels.

Concept guide **-** This Concept Guide presents an overview of plans, followed by separate chapters on each phase of the planning process. The guide includes tools, or forms, for planning and conducting various activities. It also provides background information on topics important to managing plans, such as group dynamics and statistical analysis, as well as practical information and suggestions for tailoring the process to the needs of your community. Develop a one-page tip sheet that relate to the management of plan and group dynamics. You are encouraged to copy and share the tip sheets with group facilitators and working group chairpersons.

Meeting guide **-** The Meeting Guide is intended for use when planning and conducting meetings. It contains an introduction, followed by a separate section for each phase of the planning process. For each phase, it includes meeting objectives, recommends an agenda, and suggests specific activities that can be incorporated with your own ideas.

Visual aids **-** The packet of Visual Aids includes camera, ready copy for overheads and reproducible text for handouts. Some of these materials are intended to serve as models for overheads and handouts you will need to develop to present data and other information for your own community.

Detailed implementation plan

The analysis steps make it possible to draw a Project Planning Matrix (PPM). The PPM is a matrix of four columns and four rows providing 18 squares for a comprehensive description of a project. PPM shows both the project's logical structure (the links between the inputs/activities and the objectives to be achieved under certain Assumptions), and its major quantitative data. The PPM is useful in two ways:

- Showing activities t be carried out in their sequential order
- Shows the type of activity to be done and by who
- Shows what activities should be done in what time
- Shows the budget needed for each activity

Goal: Improve health of the community in Siaya County

Objective	Activity	Time	Responsible person	Amount	Source of cash	Threats/ assumptions
To reduce the number of deaths due to diarrheal diseases from 20 per month to 15 per month by 2015	1.Distribute Chlorine to every homes	Jan	CPHO	30,000	DMOH	
	2.Educate family members in 20 homes in the use of water treatment	March	CHPO	-	-	Rain
	3.Drill 2 boreholes in the community	April	Water engineer	200,000	County treasury	Landscape
	4.Construct pit latrines in 30 homesteads	Aug	Water and sewerage office	90,000	KEWASCO	
	5.Provide ORS to children with diarrhea	Dec	Area nurse	20,000	DMOH	Stock out at KEMSA

Community Health Evaluation

- Community health evaluation is systematic studies conducted to assess how well a program is working in a community and why. Through evaluation, the implementers of community health services can:
    - Create more effective and efficient programs
    - Make better management decisions
    - Support new and innovative approaches and emerging practices
    - Continuously improve existing programs
    - Subsequently improve human health and the environment

- Input indicators – are efforts or what you invest in an activity. These can be resources e.g. money, personnel, time etc.

- Process indicators – are procedures, methods or steps followed during implementation of an activity. A process evaluation assesses whether a program or process is implemented as designed or operating as intended and identifies opportunities for improvement. Process evaluations often begin with an analysis of how a program currently operates. Process evaluations may also assess whether program activities and outputs conform to statutory and regulatory requirements, policies, and program design or customer expectations.

- Output indicators – are immediate outcome or results of an activity

Purposes of evaluation

- Answer questions about projects, policies and programs, particularly about their effectiveness and efficiency.
- Assess how well a program is working and why.
- Implementers can make better management decisions; support new and innovative approaches and emerging practices; continuously improve existing programs; and subsequently, improve human health and the environment.
- Evaluations should help to draw conclusions about five main aspects of the intervention:

    - Relevance
    - Effectiveness
    - Efficiency - Finally, cost-benefit or cost-effectiveness analysis assesses the efficiency of a program. Evaluators outline the benefits and cost of the program for comparison. An efficient program has a lower cost-benefit ratio.
    - Impact
    - Sustainability
- Information gathered provides the basis for the evaluative analysis

Types of program evaluations are there

Design - A design evaluation is conducted early in the planning stages or implementation of a program. It helps to define the scope of a program or project and to identify appropriate goals and objectives. Design evaluations can also be used to pre-test ideas and strategies.

Impact - An impact evaluation is a subset of an outcome evaluation. It assesses the causal links between program activities and outcomes. This is achieved by comparing the observed outcomes with an estimate of what would have happened if the program had not existed (e.g., would the water be swimmable if the program had not been instituted).

Cost-effectiveness - Cost-effectiveness evaluations identify program benefits, outputs or outcomes and compare them with the internal and external costs of the program. Cost-effectiveness evaluations identify program benefits, outputs or outcomes and compare them with the internal and external costs of the program. The impact evaluation determines the causal effects of the program. This involves trying to measure if the program has achieved its intended outcomes, i.e. program outcomes.

Measures of success

There are 4 essential key success measures and one auxiliary measure that are all of great value. The 4 essential key measures are:

- Financial viability - Example: profitability.
- Customer satisfaction - Example: performance on customer satisfaction surveys.
- Employee satisfaction - Example: performance on employee satisfaction surveys.
- Contribution to society - Example: number of trees saved by developing paperless processes.

These measures are important because:

- Financial viability measures organizational survival and growth.
- Customer satisfaction is important because without happy customers the organization will fail.
- Employee satisfaction is important because over the long term it is impossible to have an organization with unhappy employees that has happy customers
- Contribution to society (e.g., environment, ethics, safety, social responsibility) is important because every organization needs more than a simple profit motive to attract and retain the best talent and to sustain itself over time. An organization in which greed is a core value will ultimately devour itself.
- Key operational results are not considered an essential key success measure on the order of the first 4 and therefore are viewed as serving an auxiliary function.

Outcome evaluation

Outcome indicators – are long terms effects as a result of the outputs. Outcome evaluations

examine the results of a program (intended or unintended) to determine the reasons why there are differences between the outcomes and the program's stated goals and objectives. Outcome measurement is a systematic way to assess the extent to which a program has achieved its intended outcomes. It helps understand whether the program is effective or not.

Monitoring and Evaluation plan

Objective	**Activity**	**Indicators**	**Means of verification**
To reduce the number of deaths due to diarrheal diseases from 20 per month to 15 per month by 2015	1.Distribute Chlorine to every homes	# of homes supplied with chlorine # of homes with patients with diarrhea # of chlorines distributed per home	Household survey Household survey Distribution list
	2.Educate family members in 20 homes in the use of water treatment	# of homes covered with education on use of water treatment # of family members educated on use of water treatment # of educated family members actually treating water	Education program Education report Home visit
	3.Drill 2 boreholes in the community	# of boreholes drilled in the community # of drilled boreholes has water	Survey Survey

		# of people fetching water from the borehole daily	Survey
	4.Construct pit latrines in 30 homesteads	# of latrines constructed # of homes with latrines # of constructed latrines are in use	Construction report Household survey Household survey
	5.Provide ORS to children with diarrhea	# or ORS sachets provided # of children provided with ORS	S11 form ORS register